

Train the Trainer: health education classes for young adults experiencing homelessness

Panorama City Service Area: Village Family Services

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Support: Rebecca Deans, MD; Cynthia Santana

Introduction

- There exist barriers between unhoused youth and patient-centered, accessible health information.
- Lack of a trustworthy, firm basis in healthcare compromises quality and access for unhoused youth for years to come.
- We aim to promote health-related agency among houseless youth (participants) served by Village Family Services (VFS) through participant-driven health education.
- Recognizing that standardized curricula may miss context-specific concerns, we sought to co-create content informed directly by participants' lived experiences.

Practicum Charter

- From March 2024 to May 2025, we collaborated with staff at Village Family Services to coordinate several site visits to their temporary housing center for participants
- These visits consisted of informational interviews of both staff and participants, and eventually demo health sessions
- Community assets included inclusive meeting spaces, VFS staff time (including, but not limited to, assistance navigating health care benefits, a licensed psychotherapist, conflict resolution), local restaurants, and nearby jobs accepting applications.
- Practicum stipend resources were spent on health information pamphlets, safe sex materials, and meals for session participants.
- PDSA cycles were tracked using participant pre- and post-session surveys, along with requested topics of discussion.

Results

- On Monday, March 10, 2025, we conducted our first health education class at Village Family Services.
- We taught two engaged participants about triaging common health symptoms in different health care settings (example curricular slides below).
- Surveys administered to participants before and after the class suggest a general increase in health-related knowledge and decision-making capacity.

Plan:

- Design curricular content
- Organize logistics around holding the class (date, location, food)
- Design pre- and post-class surveys

Do:

- Held one health education class on March 10, 2025
- Two participants engaged in classes and completed pre- and post-class surveys

Study:

- Monday mornings were not an optimal time to hold classes
- Providing community resources were an essential part of the class
- Participants were eager to learn about health topics

Act:

- Assemble student leaders to continue health classes
- Identify KPSOM student organization to house project
- Identify optimal time to hold classes
- Continue to identify engaging health topics

Pulse-Check Survey Questions	Pre-Class Confidence Score (μ)	Post-Class Confidence Score (μ)
I understand the importance of seeking professional medical advice when necessary.	5/5	5/5
I can differentiate between different health care settings and understand when to go to each care setting.	4/5	4.5/5
I feel comfortable asking healthcare providers questions about my health.	4.5/5	4.5/5
I feel more confident in my ability to make informed healthcare decisions.	4/5	4.5/5
I am more confident in advocating for myself or others in a healthcare setting.	4/5	4.5/5

Your Medical Roadmap: Urgent Care vs. ER vs. PCP

Triaging Common Health Symptoms

Andrea Yeung, MS2 | Max Hukill, MS2

Alexander (they/them)

Severe pain, can't move without groaning

Alexander is 19 years old. They report abdominal pain that started around their belly button 12 hours ago. They thought it was nothing, so they ignored it. Since then, the pain migrated over to their right lower side. They haven't eaten anything in 6 hours and have no appetite. They are groaning on the exam table, and they can't move without feeling pain. The pain is 9/10. You ask them to jump up and down, and they are reluctant.

Appendicitis, torsion → Emergency Room

Next Steps

- Upcoming efforts include piloting presentation formats—such as “fact vs. myth”—to identify effective engagement strategies.
- The team will continue refining curricular content and evaluation tools. Once optimized, a guide with curricular content will be lent to Village Family Services to ensure continuity of this work.
- Given evolving affiliations, this initiative will transition to an extracurricular model, preserving its community impact while remaining responsive to participant needs.

Conclusions

- This project highlighted the importance of critical consciousness by challenging us to co-create health education *with* participants rather than *for* them, honoring their lived expertise.
- Through a lens of social accountability, we recognized our responsibility to design a scalable model that supports continuity of care beyond—and even before—health professionals’ direct involvement.
- Most importantly, building trustworthiness required transparency about our limitations, consistent presence, and deep listening – all foundational to equitable, respectful partnership.

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